

Asperger’s & Level 1 Autism: Clinical Home Strategy Guide

Managing Masking, Cognitive Fatigue & Social Reciprocity | Framework: Dr. Tony Attwood's Autism Spectrum Clinical Methodology

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Clinical Resource Packet

CHILD'S NAME	AGE / GRADE	DATE	COMPLETED BY

Clinical Support Strategies for Level 1 Autism / Asperger Profile

CHALLENGE DOMAIN	WHAT THE CHILD EXPERIENCES	RECOMMENDED CLINICAL HOME ACCOMMODATION
After-School Restraint Collapse	Autistic children often "mask" (suppress autistic traits) to fit in at school all day. When they enter the safe home, accumulated cognitive exhaustion erupts in tears or meltdowns.	Implement the "Decompression Hour": Zero questions, zero homework demands, and low sensory input for 45 minutes immediately upon arriving home.
Literal & Concrete Thinking	Metaphors, sarcasm, idioms, and implied requests ("Look at the mess on the floor") are misinterpreted or cause confusion.	Use direct, concrete language: "Please pick up your shoes and put them on the shoe rack." Give one instruction at a time.
Special Interests (Monotropism)	Intense focus on specific subjects (space, trains, coding) is not an obsession to extinguish; it is a neurological regulation tool.	Use special interests as positive bridges: incorporate favorite themes into reading exercises, maths word problems, and social conversations.
Sensory Overload Recovery	Fluorescent lighting, cafeteria clamor, and playground unpredictability overload sensory channels.	Create a designated "Sensory Sanctuary" at home: dim lighting, weighted lap pad, sound-muffling headphones, and preferred soothing textures.

Clinical Insight: Masking is exhausting. If your child behaves like an angel at school but has massive meltdowns at home, they are not being manipulative—they feel safe enough with you to release their distress.